Patient Record — ID 1683

Age	28	**Sex**	F
Race/Ethnicity	Non-Hispanic Asian	**Insurance**	Employer-sponsored commercial insurance
Total Encounters	8	**Generation Seed**	4446

Primary Diagnoses

• Aplasia cutis congenita
• Pregnancy (second trimester)
• Graves disease
• Maternal febrile illness during first trimester affecting fetus
• Migraine without aura
• Thrombocytopenia

Comorbidities

• Personal History of Cholelithiasis
• Pregnancy (second trimester)

Patient Profile

Sex	F
Age at First Encounter	28
Race/Ethnicity	Non-Hispanic Asian
Insurance	Employer-sponsored commercial insurance
Occupation	High school physical education teacher
Smoking Status	Never smoker
Alcohol Use	None; denies alcohol use during pregnancy

Allergies:

• NKDA

Chronic Conditions:

• Graves disease
• Chronic hepatitis B infection
• Migraine without aura

Home Medications:

• Propylthiouracil - 100 mg PO TID
• Prenatal vitamin - 1 tablet PO daily

Surgical History:

• None

Family History:

• Mother: Autoimmune thyroid disease (hypothyroidism)
• Father: Hypertension
• Sister: Migraine disorder

Encounters (8)

Encounter 1 — 2020-01-15 — Endocrinology & Metabolism

Encounter ID	6865	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	0
Department	Endocrinology & Metabolism	**Gen. Method**	template

Chief Complaint:

Three-month history of palpitations, heat intolerance, unintentional weight loss despite increased appetite, frequent bowel movements, and insomnia.

Full Note:

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ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Sarah Chen, MD
Department: Endocrinology & Metabolism
Visit Type: outpatient

CHIEF COMPLAINT:
Three-month history of palpitations, heat intolerance, unintentional weight loss despite increased
appetite, frequent bowel movements, and insomnia.

HISTORY OF PRESENT ILLNESS:
She also reports experiencing frequent bowel movements and difficulty sleeping.

PAST MEDICAL HISTORY:
Active Problem List:
- Graves disease
- Chronic hepatitis B infection
- Migraine without aura

PAST SURGICAL HISTORY:
- None

MEDICATIONS:
Home Medications:
- Propylthiouracil 100 mg PO TID
- Prenatal vitamin 1 tablet PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
Her mother has an autoimmune thyroid condition

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None; denies alcohol use during pregnancy. Occupation: High school
physical education teacher.

VITAL SIGNS:
Her pulse is 110 bpm
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PHYSICAL EXAMINATION:
On examination, she appears anxious and has a fine tremor of her outstretched hands. her skin is warm and moist. You note a diffuse, non-tender goiter without nodularity.

LABORATORY DATA:
Her TSH level is <0.01 IU/mL (normal: 0.4-4.0 IU/mL), free T4 is 3 ng/dL (normal: 0.9-1.7 ng/dL), and total T3 is 300 ng/dL (normal: 80-180 ng/dL). Thyroid-stimulating immunoglobulin (TSI) is present.

Encounter 2 — 2023-06-15 — Obstetrics & Gynecology

Encounter ID	6862	**Type**	procedure
Attending	Dr. Maria Rodriguez	**Order**	1
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Presents for routine fetal anatomy scan at 20 weeks gestation.

Full Note:

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ENCOUNTER NOTE
Date: 2023-06-15
Provider: Dr. Maria Rodriguez, MD
Department: Obstetrics & Gynecology
Visit Type: procedure

CHIEF COMPLAINT:
Presents for routine fetal anatomy scan at 20 weeks gestation.

HISTORY OF PRESENT ILLNESS:
The patient is a 31-year-old female with a history of Graves disease initially diagnosed in
January 2020 and chronic hepatitis B infection who returns for a routine fetal anatomy scan at 20
weeks gestation. She has been on antithyroid therapy for the past 3 years. At the time of
confirming her pregnancy, she was still taking methimazole, which she had been using consistently
for several months. Her medication was later switched to propylthiouracil at her first prenatal
visit, which occurred around 12 weeks gestation. She reports no other medication use, no
infections, and no substance use. Her first pregnancy was uncomplicated. She asks if her early
pregnancy medication could have affected the baby.

PAST MEDICAL HISTORY:
Active Problem List:
- Graves disease (diagnosed 2020-01-15)
- Chronic hepatitis B infection
- Migraine without aura

PAST SURGICAL HISTORY:
- None

MEDICATIONS:
Home Medications:
- Propylthiouracil 100 mg PO TID
- Prenatal vitamin 1 tablet PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Autoimmune thyroid disease (hypothyroidism)
- Father: Hypertension
- Sister: Migraine disorder

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None; denies alcohol use during pregnancy. Occupation: High school
physical education teacher.

PHYSICAL EXAMINATION:
The maternal exam is unremarkable.

IMAGING:
The ultrasound reveals a live, singleton fetus with normal growth parameters and normal fluid
```

volume.

Encounter 3 — 2023-08-15 — Obstetrics & Gynecology

Encounter ID	6868	**Type**	outpatient
Attending	Dr. Priya Patel	**Order**	2
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal visit to discuss appropriate weight gain and caloric intake during pregnancy.

Full Note:

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ENCOUNTER NOTE
Date: 2023-08-15
Provider: Dr. Priya Patel, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal visit to discuss appropriate weight gain and caloric intake during pregnancy.

HISTORY OF PRESENT ILLNESS:
The patient returns for routine prenatal follow-up in the context of established Graves disease
and a prior anatomy scan demonstrating aplasia cutis congenita. She has a normal prepregnancy BMI
of 22 and has gained 10 kg (22 lb) since the beginning of her pregnancy. She reports following a
balanced diet and taking her prenatal vitamins regularly. She inquires about the appropriate
amount of weight gain and caloric intake for the remainder of her pregnancy.

PAST MEDICAL HISTORY:
Active Problem List:
- Graves disease (diagnosed 2020-01-15)
- Aplasia cutis congenita (diagnosed 2023-06-15)
- Chronic hepatitis B infection
- Migraine without aura

PAST SURGICAL HISTORY:
- None

MEDICATIONS:
Home Medications:
- Propylthiouracil 100 mg PO TID

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Autoimmune thyroid disease (hypothyroidism)
- Father: Hypertension
- Sister: Migraine disorder

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None; denies alcohol use during pregnancy. Occupation: High school
physical education teacher.

PHYSICAL EXAMINATION:
Her physical examination is within normal limits. Her obstetric examination is consistent with a
24-week gestation, and fetal heart tones are normal.
```

Encounter 4 — 2023-10-15 — Obstetrics & Gynecology

Encounter ID	6863	**Type**	outpatient
Attending	Dr. Maria Rodriguez	**Order**	3
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal visit to inquire about safety of continuing basketball coaching activities.

Full Note:

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ENCOUNTER NOTE
Date: 2023-10-15
Provider: Dr. Maria Rodriguez, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal visit to inquire about safety of continuing basketball coaching activities.

HISTORY OF PRESENT ILLNESS:
The patient with Graves disease, chronic hepatitis B, and migraine returns for second-trimester
prenatal care following August and June visits. She has had an uncomplicated pregnancy thus far.
She expresses a desire to continue participating in the school's sports program, which includes
coaching basketball. She inquires about the safety of her involvement in the program.

PAST MEDICAL HISTORY:
Active Problem List:
- Graves disease (diagnosed 2020-01-15)
- Aplasia cutis congenita (diagnosed 2023-06-15)
- Pregnancy (second trimester) (diagnosed 2023-08-15)
- Chronic hepatitis B infection
- Migraine without aura

PAST SURGICAL HISTORY:
- None

MEDICATIONS:
Home Medications:
- Propylthiouracil 100 mg PO TID
- Prenatal vitamin 1 tablet PO daily

ALLERGIES:
She has no known drug allergies

FAMILY HISTORY:
- Mother: Autoimmune thyroid disease (hypothyroidism)
- Father: Hypertension
- Sister: Migraine disorder

SOCIAL HISTORY:
She is a high school physical education teacher and is accustomed to an active lifestyle.

VITAL SIGNS:
Her vital signs are within normal limits

PHYSICAL EXAMINATION:
Her physical examination is unremarkable. Her obstetric examination is consistent with a 16-week
gestation, and fetal heart tones are normal.

ASSESSMENT AND PLAN:
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She has no contraindications to exercise based on her medical and obstetric history.

Encounter 5 — 2023-12-15 — Obstetrics & Gynecology

Encounter ID	6864	**Type**	outpatient
Attending	Dr. James O'Brien	**Order**	4
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal visit with reports of increased shortness of breath when climbing stairs and mild ankle swelling by end of day.

Full Note:

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ENCOUNTER NOTE
Date: 2023-12-15
Provider: Dr. James O'Brien, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal visit with reports of increased shortness of breath when climbing stairs and mild
ankle swelling by end of day.

HISTORY OF PRESENT ILLNESS:
She returns for prenatal care. With Graves disease and chronic hepatitis B, she reports feeling
generally well but has noticed increased shortness of breath when climbing stairs and mild
swelling in her ankles by the end of the day.

PAST MEDICAL HISTORY:
Active Problem List:
- Graves disease (diagnosed 2020-01-15)
- Aplasia cutis congenita (diagnosed 2023-06-15)
- Pregnancy (second trimester) (diagnosed 2023-08-15)
- Chronic hepatitis B infection
- Migraine without aura

Her obstetric history is significant for one previous full-term normal vaginal delivery with no
complications.

PAST SURGICAL HISTORY:
- None

MEDICATIONS:
Home Medications:
- Propylthiouracil 100 mg PO TID
- Prenatal vitamin 1 tablet PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Autoimmune thyroid disease (hypothyroidism)
- Father: Hypertension
- Sister: Migraine disorder

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None; denies alcohol use during pregnancy. Occupation: High school
physical education teacher.

VITAL SIGNS:
On examination, her blood pressure is 120/75 mmHg, heart rate is 88 beats per minute, and
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respiratory rate is 16 breaths per minute

PHYSICAL EXAMINATION:
Cardiac examination reveals a grade 2/6 systolic ejection murmur heard best at the left sternal border that decreases in intensity when she sits up from a supine position. Lung fields are clear to auscultation. There is mild pitting edema in both ankles.

Encounter 6 — 2024-02-15 — Obstetrics & Gynecology

Encounter ID	6867	**Type**	outpatient
Attending	Dr. Aisha Johnson	**Order**	5
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Twenty-four-hour history of throbbing bitemporal headache accompanied by nausea and sensitivity to light.

Full Note:

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ENCOUNTER NOTE
Date: 2024-02-15
Provider: Dr. Aisha Johnson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Twenty-four-hour history of throbbing bitemporal headache accompanied by nausea and sensitivity to
light.

HISTORY OF PRESENT ILLNESS:
The patient with Graves returns for obstetrics follow-up. She reports headache is accompanied by
nausea and sensitivity to light.

PAST MEDICAL HISTORY:
Active Problem List:
- Graves disease (diagnosed 2020-01-15)
- Aplasia cutis congenita (diagnosed 2023-06-15)
- Pregnancy (second trimester) (diagnosed 2023-08-15)
- Chronic hepatitis B infection
- Migraine without aura

She has a history of migraines, which have increased in frequency since becoming pregnant. Her
medical history is otherwise unremarkable.

PAST SURGICAL HISTORY:
- None

MEDICATIONS:
Home Medications:
- Propylthiouracil 100 mg PO TID
- Prenatal vitamin 1 tablet PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Autoimmune thyroid disease (hypothyroidism)
- Father: Hypertension
- Sister: Migraine disorder

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None; denies alcohol use during pregnancy. Occupation: High school
physical education teacher.

REVIEW OF SYSTEMS:
She denies any recent head trauma, fever, neck stiffness, or visual changes.

VITAL SIGNS:
Her blood pressure is 122/78 mmHg, heart rate is 78 beats per minute, and temperature is 98.6°F
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(37°C).

PHYSICAL EXAMINATION:
Physical examination is unremarkable, with no signs of focal neurological deficits.

PLAN:
Given her pregnancy and migraine history, you consider appropriate treatment options for her acute symptoms.

Encounter 7 — 2024-03-15 — Obstetrics & Gynecology

Encounter ID	6869	**Type**	outpatient
Attending	Dr. Priya Patel	**Order**	6
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal visit with no specific complaints.

Full Note:

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ENCOUNTER NOTE
Date: 2024-03-15
Provider: Dr. Priya Patel, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal visit with no specific complaints.

HISTORY OF PRESENT ILLNESS:
Patient with Graves disease and hepatitis B returns for routine prenatal care. She has no
complaints and has had an uneventful pregnancy thus far.

PAST MEDICAL HISTORY:
Active Problem List:
- Graves disease (diagnosed 2020-01-15)
- Aplasia cutis congenita (diagnosed 2023-06-15)
- Pregnancy (second trimester) (diagnosed 2023-08-15)
- Migraine without aura (diagnosed 2024-02-15)
- Chronic hepatitis B infection

PAST SURGICAL HISTORY:
- None

MEDICATIONS:
Home Medications:
- Propylthiouracil 100 mg PO TID
- Prenatal vitamin 1 tablet PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Autoimmune thyroid disease (hypothyroidism)
- Father: Hypertension
- Sister: Migraine disorder

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None; denies alcohol use during pregnancy. Occupation: High school
physical education teacher.

REVIEW OF SYSTEMS:
She denies any history of bruising, bleeding, or anemia.

VITAL SIGNS:
Her blood pressure is 120/78 mmHg, heart rate is 82 beats per minute

PHYSICAL EXAMINATION:
Physical examination is normal with no petechiae, purpura, or hepatosplenomegaly.
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LABORATORY DATA:
Laboratory results are as follows:

- Hemoglobin: 12.2 g/dL
- White blood cell count: 7,800/mm3
- Platelets: 115,000/mm3
- Prothrombin time (PT): 12 seconds (control 11-13.5 seconds)
- Activated partial thromboplastin time (aPTT): 30 seconds (control 25-35 seconds)

Encounter 8 — 2024-05-15 — Obstetrics & Gynecology

Encounter ID	6866	**Type**	inpatient
Attending	Dr. David Kim	**Order**	7
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Presents to labor and delivery unit in active labor.

Full Note:

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ENCOUNTER NOTE
Date: 2024-05-15
Provider: Dr. David Kim, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Presents to labor and delivery unit in active labor.

HISTORY OF PRESENT ILLNESS:
The patient with Graves disease and hepatitis B presents in active labor following care for
aplasia cutis congenita and thrombocytopenia. She has been compliant with her prenatal care and
reports no complications during her pregnancy. Her first child is healthy with no chronic
illnesses.

PAST MEDICAL HISTORY:
Active Problem List:
- Graves disease (diagnosed 2020-01-15)
- Aplasia cutis congenita (diagnosed 2023-06-15)
- Pregnancy (second trimester) (diagnosed 2023-08-15)
- Migraine without aura (diagnosed 2024-02-15)
- Thrombocytopenia (diagnosed 2024-03-15)
- Chronic hepatitis B infection

PAST SURGICAL HISTORY:
- None

MEDICATIONS:
Home Medications:
- Propylthiouracil 100 mg PO TID

ALLERGIES:
She has no history of drug allergies

FAMILY HISTORY:
- Mother: Autoimmune thyroid disease (hypothyroidism)
- Father: Hypertension
- Sister: Migraine disorder

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None; denies alcohol use during pregnancy. Occupation: High school
physical education teacher.

VITAL SIGNS:
On examination, her vital signs are stable

PHYSICAL EXAMINATION:
, and the fetal heart tracing is reassuring.

LABORATORY DATA:
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Her prenatal labs were significant for positive HBsAg and HBeAg.

ASSESSMENT AND PLAN:
She delivers a healthy male infant weighing 3,400 grams via spontaneous vaginal delivery. The infant's Apgar scores are 8 and 9 at 1 and 5 minutes, respectively.